

Synthetic Payer Formulary Access Agreement

Between Demo Pharma, Inc. and Summit Care Network

Agreement ID	CTR-1004
Product	BrandAlpha (Molecule Alpha)
Comparator	BrandGamma (Molecule Gamma)
Term	April 1, 2026 through March 31, 2027
Covered segment	Summit Advantage Commercial, pharmacy benefit

Purpose of this artifact. This fictional agreement is designed to test contract-clause extraction, expected-state creation, formulary deviation detection, evidence assembly, and persona-specific next-best-action generation. All parties, products, plans, identifiers, financial terms, and events are synthetic.

PoC artifact prepared for Formulary Deviation Agent demonstration

1. Parties, scope and definitions

1.1 Parties

This Formulary Access Agreement (the "Agreement") is entered into by and between Demo Pharma, Inc. ("Manufacturer") and Summit Care Network ("Payer"). The Agreement applies only to the plan and benefit segment listed in Exhibit A.

1.2 Contracted Product

"Contracted Product" means BrandAlpha, including the package identifiers listed in Exhibit A. Product mappings outside Exhibit A are not automatically in scope.

1.3 Comparator Product

"Comparator Product" means BrandGamma for Condition X. Comparator status is evaluated within the same plan, benefit type, indication, and formulary effective period.

1.4 Contracted Tier

"Preferred Tier" means Tier 2 under the Payer tier taxonomy shown in Exhibit B. If the Payer publishes another label, the operational crosswalk in Exhibit B governs for monitoring purposes.

1.5 Utilization Management

"Utilization Management" or "UM" includes prior authorization (PA), step therapy (ST), quantity limits (QL), indication restrictions, and other access edits.

1.6 Data discrepancy vs. confirmed breach

An observed difference between the contractual expected state and a formulary data source is a potential deviation. It becomes a confirmed contractual breach only after plan applicability, dates, exclusions, source quality, implementation periods, and written evidence have been reviewed by authorized personnel.

2. Formulary access commitments

2.1 Tier commitment

During the Term, Payer shall maintain the Contracted Product on Tier 2 Preferred for the Covered Segment, subject to the exclusions and transition provisions in this Agreement.

2.2 Parity commitment

The Contracted Product shall be placed no less favorably than the Comparator Product with respect to formulary status, coverage status, PA, ST, and QL, within the Covered Segment. A difference is not a parity deviation if it results from a documented indication, benefit, product-form, or plan-design distinction permitted by this Agreement.

2.3 Coverage commitment

The Contracted Product shall remain covered for the Covered Segment. A status of not covered, excluded, non-formulary, or equivalent shall constitute a potential coverage deviation unless an approved exception applies.

2.4 Utilization-management commitment

PA is not permitted. ST is not permitted. QL is not permitted. Any additional or more restrictive requirement is a potential restriction escalation.

3. Formulary change management

3.1 Advance notification

Payer shall provide written notice of a material change affecting tier, coverage, PA, ST, QL, or comparator positioning at least 30 calendar days before the planned effective date, except where a shorter period is required by law or an urgent safety action.

3.2 Implementation grace period

A discrepancy observed within 10 calendar days after an approved formulary update may be classified as administrative lag while the Payer completes system propagation, provided written confirmation and a corrective date are supplied.

3.3 Evidence hierarchy

For investigation, the parties may consider: (a) the executed Agreement and amendments; (b) Payer-issued formulary and policy documents; (c) structured formulary datasets; (d) archived snapshots; and (e) written Payer confirmation. Conflicting evidence must be reconciled before enforcement action.

3.4 Data quality safeguards

An automated alert shall be suppressed or routed to data stewardship when a source is stale, a plan mapping is incomplete, the product mapping is ambiguous, or an effective date cannot be established.

4. Review, cure and remedies

4.1 Notice of potential deviation

Manufacturer may submit an evidence package identifying the affected plan, product, expected state, observed state, source, effective date, and applicable clause. Submission does not by itself constitute a legal determination.

4.2 Payer review

Payer shall acknowledge the inquiry within five business days and provide a substantive response within ten business days, unless the parties agree otherwise in writing.

4.3 Cure

If the parties confirm a deviation, Payer shall restore the compliant status within 15 calendar days or provide an agreed remediation plan. The cure date shall be captured in the deviation case record.

4.4 Illustrative financial reconciliation

For PoC purposes only, a confirmed uncured Critical deviation lasting more than 30 calendar days may trigger a synthetic rebate reconciliation flag. No automatic withholding, offset, or legal remedy shall be executed by the agent.

5. Governance and human oversight

5.1 Authorized decision makers

Contracting personnel determine contractual applicability and enforcement. Field roles may receive approved access guidance and account-prioritization recommendations but shall not characterize an alert as a confirmed breach.

5.2 Permitted automated actions

The agent may extract clauses, create an expected-state record, compare expected and observed access, rank potential deviations, assemble evidence, draft an inquiry, and recommend next actions. Human approval is required before external payer communication or financial action.

5.3 Audit trail

The agent shall retain an audit trail for each deviation case, including the clause relied upon, the expected and observed states, the evidence sources and retrieval dates, the classification and routing decision, any human approval, and the final disposition. Audit records shall be retained for the duration of the Term and any agreed reconciliation period.

Exhibit A - Contract scope and product identifiers

Field	Contract value
Payer ID	PAY-004
Plan ID	PLN-4001
Plan name	Summit Advantage Commercial
Line of business	Commercial
Benefit type	Pharmacy
Product ID	PRD-001
Illustrative NDC11	00011111101
Comparator product ID	PRD-003
Indication	Condition X

Exhibit B - Expected state and tier crosswalk

Attribute	Expected state	Deviation trigger
Tier	Tier 2 Preferred	Tier 3, Tier 4, non-preferred or worse
Coverage	Covered	Not covered, excluded or non-formulary
PA	Not permitted	Any PA requirement
ST	Not permitted	Any ST requirement
QL	Not permitted	Any QL requirement
Parity	No less favorable than PRD-003	Comparator has better tier, coverage or UM

Exhibit C - Approved PA criteria (synthetic)

No PA criteria are approved because PA is not permitted under this Agreement. This exhibit is fictional and is not clinical guidance.

Exhibit D - Deviation classification and routing

Observed condition	Classification	Primary routing
Expected Tier 2; actual Tier 3	Tier downgrade	Contracting + Account Director
Brand worse than BrandGamma in the same plan	Potential parity deviation	Contracting + Market Access Strategy
ST added where prohibited	Restriction escalation	Contracting + FRM + Patient Services
Covered becomes not covered	Coverage loss	Contracting + Account Director + FRM
Source stale or mapping ambiguous	Data-quality exception	Data Steward

Illustrative signatures

Demo Pharma, Inc.

Name: _____

Title: _____

Date: _____

Summit Care Network

Name: _____

Title: _____

Date: _____

Legal and data disclaimer: This is a synthetic training and demonstration artifact. It is not an MMIT export, not a payer-issued agreement, not legal advice, and not intended to represent actual coverage, clinical policy, or commercial terms.